

Severe allergy and anaphylaxis: first aid

Epinephrine (adrenaline) is the first-line emergency treatment for anaphylaxis. If the person has a prescribed auto-injector or an allergy action plan, follow it. In some places, trained responders may use stock epinephrine; follow local rules, product instructions, and dispatcher guidance.

CALL EMERGENCY SERVICES NOW

Treat this as anaphylaxis if severe symptoms appear

Call your local emergency number (911 in the US/Canada, 112 in much of Europe, 999 in the UK) if someone has a possible allergic reaction with severe symptoms. Warning signs include trouble breathing, wheeze, repeated cough, throat/tongue/lip swelling, trouble swallowing or speaking, hoarse voice, faintness, collapse, a pale or floppy child, confusion, or symptoms after a known serious allergen exposure. Severe belly pain, vomiting, or widespread hives plus other symptoms can also be a warning sign. **Do not wait for a rash before acting.**

What to do, step by step

- 1 Call for emergency help.** Put the phone on speaker if you can. Say “possible anaphylaxis” and follow dispatcher instructions.
 - 2 Use epinephrine if indicated.** Help the person use their prescribed auto-injector if they cannot do it themselves, or follow their allergy action plan/local responder rules. Most auto-injectors go into the outer middle thigh and can be used through clothing. Press and hold for the time shown on that device. Then note the time it was given.
 - 3 Position them safely.** If they feel faint, weak, or dizzy, lay them flat and raise their legs if possible. Do **not** make them stand or walk. If breathing is difficult, let them sit upright. If vomiting or unconscious but breathing, place them on their side. A pregnant person is often positioned on the left side.
 - 4 Remove the trigger only if safe and quick.** Move away from insects, stop exposure, or brush/scrape away a visible stinger if you can do it without delay. Do not delay epinephrine or the emergency call to search for a trigger.
 - 5 Watch breathing and stay with them.** Symptoms can return. If symptoms do not improve after about 5–15 minutes, or they return, a second auto-injector may be needed. Give it only if one is available and the product instructions, action plan, or dispatcher say to. Keep them under observation until emergency help takes over.
 - 6 If they become unresponsive and are not breathing normally,** call for help, start CPR, and use an AED if available.
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DO NOT DELAY EPINEPHRINE

- Antihistamines may help itching or hives, but they do **not** treat airway swelling or shock and should not replace epinephrine in anaphylaxis.
- Asthma inhalers may help wheeze for some people, but they do **not** replace epinephrine for a severe allergic reaction.
- Do not give food or drink if the person is having trouble breathing, swallowing, staying awake, or may need urgent treatment.
- Do not stand the person up or walk them around if they feel faint or weak.

Milder allergy symptoms still need watching

Itchy skin, a small area of hives, sneezing, or mild swelling without breathing trouble, throat/tongue symptoms, vomiting, faintness, or confusion may be less urgent. Follow the person's allergy plan or local medical advice, and keep watching because symptoms can worsen. If you are unsure, if symptoms spread quickly, or if severe symptoms appear, treat it as an emergency.

Prepare before an emergency

- People prescribed epinephrine should carry it as instructed and check expiry dates. Many action plans recommend having two doses available.
- Make sure family, caregivers, teachers, coworkers, and trip leaders know where the auto-injector is and how to use that specific device.
- Have a written allergy action plan when possible, especially for children, schools, camps, workplaces, and travel.
- Read food and medicine labels carefully and ask about ingredients when eating away from home if a known trigger could be present.

Sources

- NHS — Anaphylaxis (symptoms, epinephrine auto-injector use, positioning, and emergency care).
- Resuscitation Council UK — Emergency treatment of anaphylaxis (recognition, adrenaline/epinephrine, positioning, and repeat dosing guidance).
- American Academy of Allergy, Asthma & Immunology — Anaphylaxis (recognition and treatment overview).
- American Red Cross — first-aid training and resources (allergic reaction/anaphylaxis training).

Guidance reflects widely taught first aid for lay responders. Laws and training standards for helping with epinephrine vary by region — follow local training, product instructions, and emergency services.

This guide is general information, not medical advice. In an emergency, always call your local emergency number. Reuse and reprint freely.